

NURS 280 — Post-Midterm / Final Cram Sheet (Lectures 11–19) 期末速记表

UBC · Kristen Gilbert · Lifespan development + Adults + Seniors · 中英对照高频考点;请与课堂笔记核对。

L11 · CHILDHOOD THROUGH PUBERTY 第11讲 童年到青春期

Development by age 按年龄的发展

- **Birth–2:** spontaneous, reflexive response; need healthy touch; **nudity, self-exploration & self-pleasure are healthy and developmentally appropriate.** 0–2岁:自发、反射性反应;需要健康触碰;裸体、自我探索与自我抚慰健康且符合发育。
- **2–5:** learn 'my body belongs to me', types of touch, gender differences; develop gender-identity certainty & private-vs-public. 2–5岁:学“身体属于我”、触碰类型、性别差异;发展性别认同与隐私/公开。
- **5–8:** body exploration common; learn how babies are made & different family types. 5–8岁:身体探索常见;学婴儿如何产生与不同家庭类型。

Puberty hormone axis 青春期激素轴

- **Hypothalamus → GnRH → pituitary (LH, FSH) → gonads → estrogen, progesterone, testosterone, sperm & eggs → puberty.** 下丘脑→GnRH→垂体(LH、FSH)→性腺→雌激素、孕酮、睾酮、精子与卵子→青春期。

Puberty changes 青春期的变化

- **All bodies:** growth spurts, skin/sweat/odor, body hair, voice changes, moodiness, crushes. 所有身体:猛长、皮肤/出汗/体味、体毛、声音变化、情绪波动、暗恋。
- **Testosterone:** broader shoulders, erections, nocturnal emissions. **Estrogen:** wider hips, breast development, menstruation. 睾酮:肩宽、勃起、梦遗。雌激素:臀宽、乳房发育、月经。
- **Conflict:** hormones→moodiness, lack of sleep→irritability, cognitive immaturity→poor decisions, desire for independent identity. 冲突:激素→情绪化、缺觉→易怒、认知不成熟→决策差、渴望独立身份。

L12 · ADOLESCENCE 第12讲 青少年期

- Often first relationships; masturbation common (**80% penises vs 58% vaginas**). 常首次关系;自慰常见(阴茎80%对阴道58%)。
- **Sexual debut:** 84% of BC students have NOT had intercourse (AHS 2023); average debut age 17 (Tsui); most (80%) in a relationship at debut; poor school/parental attachment predicts early debut. 初次:84%BC学生未性交;平均17岁;80%初次时在恋爱;学校/亲子联结差预示更早。
- **Many overestimate peers' experience** (90% thought they'd had fewer partners than peers). 多数高估同伴经验(90%以为伴侣比同伴少)。
- Oral sex ≈ as common as intercourse; BC 17% ever had it (down from 26% in 2008). 口交≈性交常见;BC 17%曾有(从2008年26%下降)。
- **'Hook-up culture' overstated:** 85% 'non-experimenters', ongoing/monogamous. Teen pregnancy fell **37.2% (1990–2010)**. “约炮文化”被夸大;85%“非尝试者”、持续专一。青少年怀孕1990–2010降37.2%。

L13 · ADULTS 第13讲 成人

- Relationship styles: serial monogamy, polyamory, casual dating, hookups, ethical/unethical non-monogamy; **~1 in 5 have been in an open relationship.** 关系类型:连续一对一、多角恋、随意约会、约炮、(不)道德非一对一;约1/5曾处于开放关系。
- Fantasy very common (98% men / 80% women fantasize about others at least occasionally); **<1/3 act out** their biggest fantasy. Masturbation: ~2× cis men vs women last month; ~half use a vibrator. 幻想很普遍(男98%/女80%至少偶尔幻想他人);不到1/3实施最大幻想。自慰:上月顺男约为女性2倍;约半数用振动器。
- 'Foreplay' is a problematic term for non-penetrative sex. Anal sex is enjoyed by many genders (not just gay men). “前戏”是对非插入式性行为的有问题说法。肛交被许多性别享受(不只男同)。
- **Orgasm gap:** het men 95% > gay 89% > bi men 88% > lesbian 86% > bi women 66% > het women 65%. Lesbians ■ het women → **it's the type of sex, not anatomy.** 高潮差距:异男95%>男同89%>双男88%>女同86%>双女66%>异女65%。女同远高于异女→在于性方式而非解剖。
- **Dysfunction:** low/responsive desire (Brotto); pre-orgasmia (mostly cis women); erectile (cis men); sexual pain = dyspareunia / vulvodynia / vaginismus. Tx: psychotherapy, mindfulness, physio, touch, drugs. 障碍:低/反应性欲望(Brotto);尚未高潮(多顺女);勃起(顺男);性疼痛=性交痛/外阴痛/阴道痉挛。治疗:心理、正念、理疗、触碰、药物。

L14 · SEX & AGING (SENIORS) 第14讲 性与衰老(老年)

- Most seniors are sexually active; sex stays important; physical well-being is tied to sex; little research exists. 多数老年人仍有性生活;性依然重要;身体健康与性相关;研究很少。
- **Benefits:** 'use it or lose it', intimacy/touch, longer healthier lives, endorphins, exercise. 益处:“用进废退”、亲密/触碰、更长寿更健康、内啡肽、锻炼。
- **Challenges:** cis women — lower estrogen (libido, thinning tissues); cis men — lower testosterone (erectile, prostate); all — STIs, dating again, limits by family/care facilities. 挑战:顺女—雌激素降(性欲、组织变薄);顺男—睾酮降(勃起、前列腺);所有人—性病、重新约会、家人/护理机构的限制。

- **Consent & aging (VCH 2009):** basic sexual knowledge; understand consequences/risk; appropriate time/place; articulate choice/preferences/resistance; recognize a partner's distress. **Capacity matters (dementia).** 同意与衰老(VCH 2009):基本性知识;理解后果/风险;合适时间/场合;表达选择/偏好/拒绝;识别伴侣的不适。能力很重要(失智)。

L15 · SEX AND THE LAW (CANADA) 第15讲 性与法律(加拿大)

- **Age of consent = 16;** close-in-age: 14–15 if partner <5 yr older, 12–13 if <2 yr older; under 12 never. 16–17 can't consent to exploitative activity (sex work, porn, authority). 同意年龄=16;年龄相近:14–15岁对方大不到5岁、12–13岁对方大不到2岁;12岁以下永不能。16–17岁不能同意剥削性活动(性交易、色情、权威关系)。
- **R v Ewanchuk (1998/99):** defined consent — **no 'implied consent';** must be affirmative & ongoing. R v Ewanchuk(1998/99):界定同意——无“默示同意”;须明确且持续。
- **Intimate images:** sharing without consent is illegal; lawful image of <18 only if private + all consent to image + consent to the activity. 私密影像:未经同意分享违法;未满18者合法影像须:私密+所有人同意拍摄+同意所拍内容。
- **Sexual assault = non-consensual sexual touching; the Crown (not the victim) decides** on charges. Globe & Mail 'Unfounded': ~1 in 5 reports dismissed as baseless. 性侵犯=未经同意的性触碰;由检方(非受害者)决定是否起诉。《环球邮报》“无依据”约五分之一报案被判无依据。
- **Sex work — Bedford (2013) + PCEPA (2014):** selling sex is legal; **buying, communicating to buy, third-party profiting, procuring & advertising are illegal** ('end-demand'). 性工作 — Bedford(2013)+PCEPA(2014):出售性服务合法;购买、为购买沟通、第三方获利、拉皮条、广告均非法(“打击需求”)。

L16 · SEXUALLY EXPLICIT MATERIAL (PORN) 第16讲 色情材料

- **Common:** past month 91.5% men / 60.2% women; lifetime 98.9% / 91.9%. Impact on sex life: 62% none, 25% positive, 11% mixed, 2% only negative. 普遍:上月男91.5%/女60.2%;一生98.9%/91.9%。对性生活影响:62%无、25%正面、11%好坏参半、2%仅负面。
- **Problems:** body misrepresentation, how pleasure is shown, violence, performer exploitation. **Benefits:** explore pleasure/fantasy/identity, healthy masturbation, partner connection. 问题:身体失真、快感呈现方式、暴力、演员剥削。益处:探索快感/幻想/身份、健康自慰、伴侣连接。
- **Myths:** 'porn addiction' is NOT a recognized diagnosis (DSM-5/ICD-11); porn doesn't ruin relationships (no link; may raise desire for partner), doesn't cause ED (linked to MORE arousal), doesn't 'rewire' the brain. 迷思:“色情成瘾”非公认诊断(DSM-5/ICD-11);色情不会毁关系(无关联;或增加对伴侣的欲望)、不致勃起障碍(与更强唤起相关)、不会“重塑”大脑。
- **Ethical porn (Ley):** legal; respects & pays performers; shows fantasy + real sex; diverse; no 'right/wrong' way; treats performers & viewers as free, consenting, powerful. 伦理色情(Ley):合法;尊重并付酬演员;既有幻想也有真实的性;多元;不设“对错”;把演员与观众当作自由、同意、有力量的个体。

L17 · GENDER 第17讲 性别

- We 'know' our gender via **bodies** (assigned at birth from genitals — data, not identity), **socialization** (from birth), and an **internal sense of self** (solidifies 18–36 months; can change across life). 我们通过身体(出生时依生殖器官指定——线索而非身份)、社会化(自出生起)和内在自我感(18–36个月稳固;一生可变)来“知道”性别。
- Options: woman, man, **nonbinary**/genderqueer/fluid, **trans**, femme/masc, **Two Spirit** (Indigenous). In Canada, expressing your gender is a right. 选项:女、男、非二元/酷儿/流动、跨性别、femme/masc、双灵(原住民)。在加拿大,表达性别是权利。
- **Transition:** social (name, pronouns, dress) and gender-affirming clinical care (hormones, upper/lower surgery). Not everyone transitions. 过渡:社会过渡(名字、代词、着装)与性别肯定临床护理(激素、上/下身手术)。并非人人过渡。
- Gender identity/expression is a protected ground under the **Canadian Human Rights Act, Criminal Code & BC Human Rights Code**. 性别认同/表达是加拿大人权法、刑法典与BC人权法下受保护事由。

L18 · QUEER IDENTITIES 第18讲 酷儿身份

- Identities: **2SLGBTQIA+** — lesbian, gay, bisexual, trans, queer, two spirit, intersex, asexual, demisexual, pansexual, questioning, MSM. 身份:2SLGBTQIA+——女同、男同、双性恋、跨性别、酷儿、双灵、间性、无性恋、半性恋、泛性恋、探索中、MSM。
- Canadian timeline: 1969 decriminalized (21+); **1973 removed from DSM**; 1996 orientation added to CHRA; **2005 same-sex marriage** (2002 QC); 2017 PM apology; 2018 Expungement Act. 加拿大时间线:1969除罪(21+);1973从DSM删除;1996性取向加入人权法;2005同性婚姻(魁省2002);2017总理道歉;2018撤销法。
- Intersex ≈ as common as **red hair**. BC youth: 75% of trans youth self-harmed in the past year, ~1/3 attempted suicide; gay/bi males ~2× and lesbian/bi females 2–3× more likely to self-harm. 间性≈与红发一样常见。BC青年:75%跨性别青年过去一年自伤;约1/3尝试自杀;男同/双性恋男约2倍、女同/双性恋女2–3倍更易自伤。
- **Protective:** GSAs & anti-bullying policies (lower discrimination/suicidality for ALL students), gender-neutral bathrooms, school connectedness, friendly symbols. A GSA (~\$1,000/yr) can save ~\$183,000 in healthcare.

保护因素:GSA与反欺凌政策(降低所有学生的歧视/自杀倾向)、性别中立卫生间、学校归属感、友善标识。GSA(约每年1000元)可节省约18.3万元医疗费。

L19 · SEX & DISABILITY 第19讲 性与残障

• Youth with disabilities are **as sexual as** peers & entitled to sexual health education (PHAC 2014); they're wrongly assumed **asexual**. SIECUS (2001): right to education, sexual health care & chances to socialize/express; train caregivers.

残障青少年与同龄人一样有性,且有权获得性健康教育(PHAC 2014);常被误认为无性。SIECUS(2001):有权获得教育、性健康护理与社交/表达机会;培训照顾者。

• **Best practices (Hingsburger):** correct info & body-part names; teach privacy; types of touch; **boundaries & the right to say 'no'** (they're often taught to comply). Build refusal/consent, body-care & safer-sex skills via task analysis, chunking, social stories. Resources: GF Strong, PleasureABLE manual.

最佳实践(Hingsburger):正确信息与身体部位名称;教隐私;触碰类型;界限与说“不”的权利(他们常被教服从)。用任务分解、分块、社会故事培养拒绝/同意、身体护理与更安全性行为技能。资源:GF Strong、PleasureABLE手册。

• **Abuse risk ↑↑:** adults with developmental disabilities 4–10× (Sobsey 1994); children 2×+ (Little 2004); women 2×+; ~80% women & 60% men molested by 18, only **3% of attackers jailed** (Hingsburger 2002). Why: care dependence, communication barriers, taught to comply, targeted, often not believed.

受虐风险↑↑:发育性残障成人4–10倍(Sobsey 1994);儿童2倍+(Little 2004);女性2倍+;约80%女性、60%男性18岁前遭骚扰,仅3%施害者入狱(Hingsburger 2002)。原因:依赖护理、沟通障碍、被教服从、成为目标、常不被相信。

READING · A KID'S FIRST BOOK ABOUT SEX 读物 ·

儿童的第一本性教育书

• Joani Blank: a pleasure-positive children's book about **feelings, not reproduction**; names the clitoris; normalizes masturbation & privacy; inclusive of any partner. An example of comprehensive, pleasure-inclusive education (Dailey's Sensuality circle; Dutch model).

Joani Blank:以快感为本、讲感受而非生殖的儿童读物;命名阴蒂;把自慰与隐私正常化;对任何伴侣包容。是全面、含快感教育的范例(戴利感官愉悦环;荷兰模式)。